TinyBolus Formulary

Peer-review document

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Source: github.com/SGP-Prime/tinybolusformulas

Privacy policy: sgp-prime.github.io/tinybolus-site/privacy.html

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This is a reference tool, not a prescribing system. Verify every value against your hospital protocols, product labelling, and the individual patient in front of you before administration.

About this document

This is the human-readable export of the TinyBolus paediatric reference formulary, organised section-by-section as it appears in the application. Each item is presented with its dose specification, applicable age brackets (where present), route, clinical notes, and numerical references pointing into the bibliography at the end of the document.

How to read each item

Items fall into a small number of shapes that map directly onto how the application computes them:

- **Single dose** (e.g., 10 µg/kg): one factor multiplied by weight, often with a hard maximum cap.
- **Dose range** (e.g., 2–3 mg/kg): a low–high range, also weight-scaled, optionally with a maximum cap on either bound.
- **Age-bracketed**: different dose factors for different age cohorts. Rendered as a small table per item.
- **Age-bracketed fixed value**: not weight-scaled, just a string per age band (e.g., laryngoscope blade size).
- **Computed**: piecewise logic that doesn't reduce to a single formula (ETT sizing, fluid maintenance, LMA / supraglottic airway sizing, vital-sign normal ranges).

Age brackets

Bracket bounds are printed exactly as the application resolves them. A bound in *months* is inclusive in completed months ("≤12 mo" includes a child of exactly 1 y 0 m). A bound in *years* covers every child of that age in completed years ("1–6 y" runs through 6 y 11 m). Where a months bracket ends on a birthday, the next bracket starts one month later and is marked ">": ">1–6 y" begins at 1 y 1 m. "≥6 mo" starts at exactly 6 months. Weight bands are inclusive on both bounds.

Visibility

This document prints every row in the formulary. The app shows a curated default set and keeps the remaining rows behind a switch in Settings; those rows are tagged *hidden by default* beside their name below. A hidden row is still maintained, sourced and reviewed — it is simply off until a user enables it.

Hard floors

Some items carry an *age* or *weight* floor below which the formulary declines to suggest a dose. In the app this renders as ∅ (empty-set glyph) with a "not indicated" explanation; the same wording is used here. A floor records where the cited source stops (no data, not recommended, outside the licensed indication) — it is not a claim of formal contraindication. Items with such floors are flagged in red below each row.

References

Every dose / range / age bracket / sizing rule carries one or more citations. References are numbered in encounter order throughout this document, with a full bibliography at the end. The numbers in [brackets] after each item refer to entries in that bibliography.

Source

The formulary itself lives at github.com/SGP-Prime/tinybolusformulas as a single machine-readable JSON file. This document is regenerated from that file on demand by `tool/generate_formulary_review_html.dart`; do not hand-edit it.

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EMERGENCY — PALS

Compression rate

VALUE `100–120/min`

References: [1]

Compression depth

RULE `4 cm <1 y · 5 cm ≥1 y`

not exceeding 6 cm at any age

References: [1]

C:V ratio

VALUE `15:2`

15:2 if paediatric-BLS trained · 30:2 otherwise

References: [1]

Defibrillation

DOSE `4 J/kg`

refractory VF/pVT (6th shock on): stepwise to 8 J/kg, max 360 J · adolescent 120–200 J

References: [1], [2]

Cardioversion (synchronised)

DOSE `1 J/kg`

double per attempt to max 4 J/kg

References: [1]

Adrenaline (cardiac arrest)

IV/IO

DOSE `10 µg/kg (max 1 mg)`

repeat every 3–5 min · max 1 mg/dose

References: [1], [3]

Amiodarone

IV/IO

DOSE `5 mg/kg (max 300 mg)`

after 3rd shock · repeat after 5th · max 150 mg subsequent

References: [1]

Atropine

IV

DOSE `20 µg/kg (max 500 µg)`

References: [1], [4], [5]

Adrenaline (bradycardia)

IV/IO

DOSE `1–2 µg/kg`

unresponsive to oxygenation/ventilation · atropine if vagal

References: [1], [4]

Adenosine

IV

DOSE `1st 0.1 mg/kg (max 6 mg) · 2nd 0.3 mg/kg (max 18 mg)`

2nd dose ≥1 min later · then q1–2 min, +0.05–0.1 mg/kg a dose, max 0.5 mg/kg · cardiology

References: [1]

Magnesium sulphate

HIDDEN BY DEFAULT

IV/IO

DOSE `50 mg/kg (max 2 g)`

References: [1]

Vital Signs

Heart Rate

≤1 mo	100–205 bpm
2–12 mo	100–180 bpm
>1–2 y	98–140 bpm
3–5 y	80–120 bpm
6–11 y	75–118 bpm
≥12 y	60–100 bpm

References: [6]

Systolic BP

≤1 mo	67–84 mmHg
2–12 mo	72–104 mmHg
>1–2 y	86–106 mmHg
3–5 y	89–112 mmHg
6–9 y	97–115 mmHg
10–11 y	102–120 mmHg
12–15 y	110–131 mmHg
≥16 y	90–140 mmHg

References: [6]

Diastolic BP

≤1 mo	35–53 mmHg
2–12 mo	37–56 mmHg
>1–2 y	42–63 mmHg
3–5 y	46–72 mmHg
6–9 y	57–76 mmHg
10–11 y	61–80 mmHg
12–15 y	64–83 mmHg
≥16 y	60–90 mmHg

References: [6]

Respiratory Rate

≤12 mo	30–53 /min
>1–3 y	22–37 /min
4–5 y	20–28 /min
6–12 y	18–25 /min
≥13 y	12–20 /min

References: [6]

Tidal Volume

DOSE `6–8 mL/kg`

per IDEAL body weight — if obese use estimated weight · lower end for neonates

References: [7], [8], [1]

Induction

Propofol

IV

≤1 mo	1–2.5 mg/kg
2–12 mo	3–4 mg/kg
>1–6 y	3–3.5 mg/kg
7–12 y	2.5–3 mg/kg
≥13 y	2–2.5 mg/kg

≈ halve after sevoflurane induction

References: [9], [10], [11], [12]

Thiopental

HIDDEN BY DEFAULT

IV

≤1 mo	3–4 mg/kg
2–12 mo	5–8 mg/kg
>1 y	5–6 mg/kg

then 1 mg/kg as needed · titrate to effect

References: [13]

Etomidate

HIDDEN BY DEFAULT

IV

DOSE `0.2–0.3 mg/kg`

for haemodynamic stability

References: [14], [15]

Ketamine

IV

DOSE `1–2 mg/kg`

References: [16], [14]

Ketamine (IM)

HIDDEN BY DEFAULT

IM

DOSE `4–5 mg/kg`

onset 3–5 min

References: [16], [14]

Fentanyl (induction)

IV

DOSE `1–3 µg/kg`

References: [17], [14], [18]

Midazolam

IV

≤6 mo	0.05–0.1 mg/kg — *<6 mo: prolonged effect · slow injection · hypotension with opioids*
7 mo – 5 y	0.05–0.1 mg/kg
≥6 y	0.025–0.05 mg/kg

CAP `max 5 mg`

References: [19], [14]

Midazolam (oral premed)

HIDDEN BY DEFAULT

PO

DOSE `0.5 mg/kg (max 20 mg)`

References: [14]

Midazolam (buccal premed)

HIDDEN BY DEFAULT

BUCCAL

DOSE `0.3 mg/kg (max 10 mg)`

if oral refused

References: [14]

Midazolam (IN premed)

HIDDEN BY DEFAULT

IN

DOSE `0.2–0.3 mg/kg (max 10 mg)`

5 mg/mL — divide between nostrils

References: [20], [14]

Dexmedetomidine (IN premed)

HIDDEN BY DEFAULT

IN

DOSE `1–3 µg/kg (max 200 µg)`

100 µg/mL solution · onset ~30 min

References: [21], [22]

Rocuronium

IV

DOSE `0.6–1.2 mg/kg`

upper range for RSI

References: [23], [24]

Succinylcholine

HIDDEN BY DEFAULT

IV

≤12 mo	2–3 mg/kg — *raised in infants — larger extracellular volume*
>1 y	1–2 mg/kg

intubating dose · avoid in malignant hyperthermia / hyperkalaemia risk

References: [24], [15], [25]

Atracurium

HIDDEN BY DEFAULT

IV

≤12 mo	0.3–0.4 mg/kg
>1 y	0.4–0.5 mg/kg

References: [14], [15]

PONV

Ondansetron

IV

DOSE `0.1 mg/kg (max 4 mg)`

Not indicated <1 months (age floor)

References: [26], [14], [27]

Dexamethasone (PONV)

IV

DOSE `0.15–0.25 mg/kg (max 4 mg)`

References: [26], [28], [14]

Droperidol (PONV)

HIDDEN BY DEFAULT

IV

DOSE `10–50 µg/kg (max 1.25 mg)`

Not indicated <24 months (age floor)

2nd line · avoid in long QT · give 30 min before end of surgery

References: [29], [30]

Painkillers

Paracetamol

IV

≤10 kg 7.5 mg/kg

>10 kg 15 mg/kg

CAP `max 1 g`

q6h (min 4 h) · max 4 doses/24 h

References: [31], [14], [32]

Ketorolac

IV

DOSE `0.5 mg/kg (max 30 mg)`

Not indicated <6 months (age floor)

References: [33], [14]

Morphine

IV

≤1 mo 0.025–0.05 mg/kg

≥2 mo 0.1 mg/kg

References: [34], [14]

Fentanyl (analgesia)

HIDDEN BY DEFAULT

IV

DOSE `0.5–1 µg/kg`

References: [14], [35], [18]

Metamizole

HIDDEN BY DEFAULT

IV

DOSE `8–16 mg/kg (max 1 g)`

Not indicated <3 months (age floor)

References: [36], [37]

Ibuprofen (oral)

HIDDEN BY DEFAULT

PO

DOSE `5–10 mg/kg (max 400 mg)`

Not indicated <3 months (age floor)

not <5 kg

References: [38], [14], [39]

Tramadol

HIDDEN BY DEFAULT

IV/PO

DOSE `1–2 mg/kg (max 100 mg)`

Not indicated <12 months (age floor)

q6h · respiratory depression caution

References: [40], [41]

Diclofenac (PO/PR)

HIDDEN BY DEFAULT

PO/PR

DOSE `0.3–1 mg/kg`

Not indicated <12 months (age floor)

q8–12h

References: [42], [41]

Emergency Drugs

Adrenaline (anaphylaxis IM)

IM

DOSE `10 µg/kg (max 500 µg)`

repeat every 5 min as needed · auto-injectors: 150 µg 1–5 y · 300 µg 6–12 y · 500 µg >12 y

References: [1], [4]

Adrenaline (anaphylaxis IV)

IV

DOSE `1 µg/kg`

titrate to response · IM adrenaline is first-line

References: [1]

Adrenaline (anaphylaxis infusion)

HIDDEN BY DEFAULT IV

DOSE `0.5–1 mL/kg/h`

1 mg in 100 mL NaCl 0.9% = 10 µg/mL · dedicated pump line · after 2 failed IM doses

References: [43], [4]

Clemastine

IV

DOSE `25 µg/kg (max 2 mg)`

Not indicated <12 months (age floor)

second-line, after adrenaline

References: [44], [45], [46]

Atropine

IV

DOSE `20 µg/kg (max 500 µg)`

References: [1], [4], [5]

Ephedrine

IV

≤6 mo 0.6–1.2 mg/kg — *far higher than older-child dose – indirect agent, immature sympathetics*

≥7 mo 0.1–0.3 mg/kg

ampoule 30 mg/mL — dilute

References: [47], [14], [15]

Phenylephrine

IV

DOSE `1–20 µg/kg (max 500 µg)`

upper range for tet spells · hypotension: start low, titrate

References: [14], [15]

Lidocaine

IV

DOSE `1 mg/kg (max 100 mg)`

antiarrhythmic — refractory VF / pulseless VT

References: [1], [3]

Glucose 10%

IV/IO

DOSE `2 mL/kg`

hypoglycaemia · = 0.2 g/kg · recheck glucose after

References: [1], [4]

Glucagon

HIDDEN BY DEFAULT

IM/SC

≤25 kg 0.5 mg

>25 kg 1 mg

hypoglycaemia · no response after 10 min: repeat or IV glucose

References: [48]

Calcium gluconate 10%

IV/IO

DOSE `0.5 mL/kg (max 20 mL)`

hypocalcaemia · hyperkalaemia (ECG changes; not in arrest) · Ca-blocker overdose · slow IV

References: [1], [49]

Sodium bicarbonate 8.4%

IV/IO

DOSE `1 mmol/kg`

1 mL/kg of 8.4% · tricyclic toxicity / severe acidosis · not routine in arrest

References: [1], [49]

Dantrolene

IV

DOSE `2.5 mg/kg`

reconstitute NOW (volume per vial label) · repeat 1 mg/kg q5 min · may exceed 10 mg/kg

References: [50], [51]

Midazolam (buccal, status epilepticus)

HIDDEN BY DEFAULT

BUCCAL

≤11 mo	2.5 mg
1–4 y	5 mg
5–9 y	7.5 mg
≥10 y	10 mg

Not indicated <3 months (age floor)

seizure ≥5 min · 3–6 mo: hospital only · repeat once after 5–10 min

References: [52], [1]

Midazolam (IV, status epilepticus)

IV/IO

DOSE `0.15 mg/kg (max 10 mg)`

repeat once · off-label use

References: [53], [1]

Levetiracetam (status epilepticus)

HIDDEN BY DEFAULT

IV/IO

DOSE `40–60 mg/kg (max 4.5 g)`

Not indicated <1 months (age floor)

loading dose · 2nd line after 2 benzodiazepine doses · give over 5 min · off-label

References: [1], [53]

Insulin + glucose (hyperkalaemia)

IV

DOSE `0.1 units/kg (max 10 units)`

WITH glucose 10% 5 mL/kg (max 250 mL) — never insulin alone · check glucose & K+ q15min

References: [54], [1]

Salbutamol (nebulized, hyperkalaemia)

HIDDEN BY DEFAULT NEBULIZED

VALUE `2.5–5 mg`

2.5 mg ≤25 kg · 5 mg >25 kg · repeat ≤5× · with insulin–glucose

References: [54], [1]

Fluids and Blood

Fluid Bolus

IV

DOSE `10 mL/kg (max 500 mL)`

balanced crystalloid · cardiogenic shock: 5 mL/kg

References: [1], [4]

Maintenance

RULE `Holliday-Segar 4-2-1: 4 mL/kg/h (first 10 kg) · 2 mL/kg/h (next 10 kg) · 1 mL/kg/h thereafter`

4-2-1 (Holliday-Segar) · isotonic + glucose ± K+ · <1 mo: neonatal regimen

References: [55], [56]

Total Blood Volume

HIDDEN BY DEFAULT

≤1 mo	85 mL/kg
2 mo – 2 y	75 mL/kg
≥3 y	70 mL/kg

References: [15]

Tranexamic Acid

IV

DOSE `15 mg/kg (max 1 g)`

loading dose over 10 min · then 2 mg/kg/h for ≥8 h

References: [57], [58], [59], [1]

Max allowable blood loss

HIDDEN BY DEFAULT

VALUE `EBV × (Hct – min Hct) / Hct`

estimate, not a transfusion trigger · blood volume ~70–100 mL/kg · source uses mean Hct

References: [60], [61]

Equipment

ETT

RULE `Cuffed: 3.5 + age/4, rounded to ±0.5`

<1 MO `Neonatal weight table (≤4 kg, uncuffed; heavier neonates use the infant age table):`

≤1.5 kg · preterm ≤32 wk	size 2.5
≤3 kg · 33–39 wk	size 3.0
≤4 kg · term ≥39 wk	size 3.5

INFANT `age table (cuffed):`

≤6 mo	size 3.5
7–12 mo	size 4.0

cuffed sizes — uncuffed add 0.5 (≥1 y) · neonatal table: uncuffed sizes

References: [62], [63]

ETT insertion depth

RULE `6 + weight (<1 mo) · weight/2 + 8 (<1 y) · age/2 + 12 (≥1 y)` *(cm)*

nasal: +1 cm <1 y · +2–3 cm ≥1 y

References: [64], [65], [49]

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iGel LMA

IGEL SIZING `weight bands (inclusive on both bounds):`

2–5 kg	size 1
5–12 kg	size 1.5
10–25 kg	size 2
25–35 kg	size 2.5
30–60 kg	size 3
50–90 kg	size 4
90–∞ kg	size 5

References: [66], [67]

AuraGain LMA

HIDDEN BY DEFAULT

AURAGAIN SIZING `weight bands (inclusive on both bounds):`

2–5 kg	size 1
5–10 kg	size 1.5
10–20 kg	size 2
20–30 kg	size 2.5
30–50 kg	size 3
50–70 kg	size 4
70–100 kg	size 5
100–∞ kg	size 6

References: [68]

Laryngoscope blade

≤1 y	Miller 0–1
2–7 y	Macintosh 2
≥8 y	Macintosh 3

Miller lifts the epiglottis · Macintosh 1 equally effective

References: [69], [15]

Face mask

≤1 mo	Size 0
2–12 mo	Size 1
>1–3 y	Size 2
4–8 y	Size 3
9–14 y	Size 4
≥15 y	Size 5

cover nose to chin cleft — anatomical fit over size

References: [15], [70]

Reservoir bag

≤6 mo	0.5 L
7 mo – 6 y	1 L
≥7 y	2 L

References: [71], [72]

Suction catheter

HIDDEN BY DEFAULT

≤1 mo	6 Fr
2 mo – 1 y	8 Fr
2–8 y	10 Fr
≥9 y	12–14 Fr

References: [49], [15]

Nasogastric / orogastric tube

HIDDEN BY DEFAULT

≤1 mo	6 Fr
2 mo – 1 y	8 Fr
2–8 y	10 Fr
≥9 y	12–14 Fr

larger sizes for adolescent decompression

References: [49], [15]

Oral / nasal airway sizing

HIDDEN BY DEFAULT

VALUE `by landmark`

Guedel: incisors → jaw angle · nasal: nostril → tragus −1 cm; not in basal-skull fracture

References: [73], [74], [75]

Respiratory

Salbutamol (inhaled)

INHALED

≤5 y 2–10 puffs

≥6 y 4–10 puffs

100 µg/puff · repeat incrementally as needed

References: [76], [77]

Salbutamol (nebulized)

HIDDEN BY DEFAULT NEBULIZED

≤5 y 2.5 mg

≥6 y 5 mg

References: [76], [77]

Ipratropium (nebulized)

HIDDEN BY DEFAULT NEBULIZED

≤11 y 250 µg

≥12 y 500 µg

References: [14]

Magnesium sulphate (asthma)

IV

DOSE `40 mg/kg (max 2 g)`

infuse over 20 min

References: [78], [14], [1]

Adrenaline (nebulized)

NEBULIZED

DOSE `0.5 mg/kg (max 5 mg)`

croup / stridor · 1:1000 (1 mg/mL)

References: [79], [14]

Dexamethasone (anti-inflammatory)

HIDDEN BY DEFAULT

IV

DOSE `0.5 mg/kg (max 8 mg)`

airway oedema / post-extubation stridor

References: [80], [81]

Propofol (laryngospasm)

IV

DOSE `0.5 mg/kg`

give in increments

References: [82]

Succinylcholine (laryngospasm)

IV

DOSE `0.1–0.5 mg/kg`

lower end may spare breathing · avoid in malignant hyperthermia / hyperkalaemia risk

References: [83], [82]

Succinylcholine (IM)

IM

DOSE `4 mg/kg (max 200 mg)`

laryngospasm or no IV access · avoid in malignant hyperthermia / hyperkalaemia risk

References: [83], [82]

Rocuronium (laryngospasm)

IV

DOSE `0.6–1.2 mg/kg`

when succinylcholine contraindicated (malignant hyperthermia) — then secure the airway

References: [24], [23]

Salbutamol (IV, severe asthma)

HIDDEN BY DEFAULT

IV

DOSE `15 µg/kg (max 250 µg)`

bolus over 10 min · infusion 1–2 µg/kg/min (>2: ICU) · monitor K+ · off-label <12 y

References: [76], [84]

Hydrocortisone (IV, asthma)

HIDDEN BY DEFAULT

IV

DOSE `4 mg/kg`

q4h — if oral not tolerated

References: [76]

Reversal Agents

Naloxone (titrated)

IV

DOSE `10 µg/kg`

titrate to respiration · repeat q2–3 min

References: [14], [1]

Naloxone (overdose)

IV/IO/IM

DOSE `0.1 mg/kg (max 2 mg)`

References: [1], [14]

Flumazenil

IV

DOSE `10 µg/kg (max 200 µg)`

repeat ≤4× · total max 50 µg/kg (up to 1 mg) · licensed ≥1 y

References: [14]

Sugammadex

IV

DOSE `2–4 mg/kg`

upper dose for deep block

References: [85], [86]

Sugammadex (profound block)

HIDDEN BY DEFAULT

IV

DOSE `16 mg/kg`

can't intubate / can't oxygenate: reverses full-dose rocuronium · off-label in children

References: [87], [88]

Neostigmine

HIDDEN BY DEFAULT

IV

DOSE `0.04–0.07 mg/kg (max 5 mg)`

with atropine · slow over 1–2 min, after first recovery signs

References: [14], [15]

Atropine (with neostigmine)

HIDDEN BY DEFAULT

IV

DOSE `20 µg/kg (max 500 µg)`

with or just before neostigmine

References: [14], [15]

Neostigmine + glycopyrronium

HIDDEN BY DEFAULT

IV

DOSE `0.02 mL/kg (max 2 mL)`

References: [89]

Regional Anesthesia

Lidocaine (perineural)

PERINEURAL

≤5 mo maximum 3.5 mg/kg

≥6 mo maximum 5 mg/kg

plain solution — no adrenaline

References: [15], [14]

Bupivacaine (perineural)

PERINEURAL

≤5 mo maximum 1.5–2 mg/kg

≥6 mo maximum 2–2.5 mg/kg

infusion: 0.1–0.3 mg/kg/h

References: [90], [91]

Ropivacaine (perineural)

PERINEURAL

≤5 mo maximum 2 mg/kg

≥6 mo maximum 3 mg/kg

infusion: 0.1–0.3 mg/kg/h

References: [92], [93], [91]

Intralipid 20% bolus

IV

DOSE `1.5 mL/kg`

LA toxicity rescue · repeat if unstable · max total 12 mL/kg

References: [90], [94]

Intralipid 20% infusion

IV

DOSE `0.25 mL/kg/min`

LA toxicity rescue – continue ≥15 min after stability

References: [90], [94]

Levobupivacaine (max)

PERINEURAL

DOSE `maximum 1.25 mg/kg (max 150 mg)`

Not indicated <6 months (age floor)

SmPC figure: ilioinguinal block, per side · no data <6 mo

References: [95]

Weight Estimation

Used when the user does not supply a measured weight. <1 month and >12 years always require user input.

1–12 mo (infant)

FORMULA `weight (kg) = (0.5 × months) + 4`

References: [49]

1–5 y (young child)

FORMULA `weight (kg) = (2 × age) + 8`

References: [49]

6–12 y (school-age)

FORMULA `weight (kg) = (3 × age) + 7`

References: [96], [49]

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Citations numbered in order of first encounter throughout this document.

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